Acute Stress Disorder (DSM-5-TR): Teaching Overview

Clinical Companion Knowledge Library: for counselors and counseling students

1. What it is

Acute stress disorder (ASD) is a cluster of intense trauma reactions (intrusive memories, negative mood, dissociation, avoidance, and arousal) that begins after a traumatic event and lasts from **3 days to 1 month** (American Psychiatric Association [APA], 2022). If symptoms continue past a month, PTSD is considered instead. It belongs to DSM-5-TR's "Trauma- and Stressor-Related Disorders" chapter.

Stress reactions are common. In a national survey 3 to 5 days after the September 11, 2001 attacks, 44% of U.S. adults reported at least one substantial stress symptom, even far from the attacks, and 35% of parents said their children had stress symptoms (Schuster et al., 2001). Most acute reactions fade without becoming a disorder.

A limited predictor of PTSD. Across 22 studies, an ASD diagnosis predicted later PTSD reasonably well in those who had it, but most people who went on to develop PTSD had **not** met ASD criteria. The author concluded that describing the full range of early stress reactions is more useful than relying on ASD to predict PTSD (Bryant, 2011).

Counselor framing: early after trauma, the goals are safety, practical support, calm, and connection, along with watching for people who are not recovering. Most people recover with time and support.

2. Core symptoms to watch for

DSM-5-TR criteria, paraphrased (APA, 2022); use the manual itself for formal diagnosis.

- **Exposure** to actual or threatened death, serious injury, or sexual violence: directly experiencing it, witnessing it, learning it happened to a close family member or friend, or repeated exposure to aversive details (as for first responders).
- **9 or more of 14 symptoms** from five categories, beginning or worsening after the event:
  - **Intrusion:** unwanted distressing memories; distressing dreams; flashbacks; intense or prolonged distress or physical reactions to reminders.
  - **Negative mood:** persistent inability to feel positive emotions such as happiness, satisfaction, or love.
  - **Dissociation:** an altered sense of reality (feeling in a daze, time slowing); inability to remember an important part of the event.
  - **Avoidance:** efforts to avoid distressing memories, thoughts, or feelings; efforts to avoid external reminders (people, places, conversations, activities, objects).
  - **Arousal:** sleep disturbance; irritability or angry outbursts; hypervigilance; poor concentration; exaggerated startle.
- **Duration:** 3 days to 1 month after the trauma, with clinically significant distress or impairment, not due to a substance, a medical condition (such as a mild head injury), or brief psychotic disorder.

Rule out: adjustment disorder (the stressor does not meet the trauma criterion, or the full symptom pattern is absent), PTSD (after 1 month), panic attacks, a head injury, and substance effects. Normal grief is not ASD (APA, 2022).

Safety and referral (general clinical consensus): ensure physical safety and medical care first. Ask about suicidal thoughts, heavy alcohol or drug use to cope, and ongoing danger (such as domestic violence). Refer for evaluation when symptoms are severe, dissociation is marked, or functioning has collapsed, and re-screen at about 1 month for PTSD.

3. Frequently used assessment instruments

These are among the most widely used measures for screening, rating severity, and tracking progress in acute stress disorder. **Screening tools do not diagnose;** a positive screen calls for a clinical interview against DSM-5-TR criteria. Click an instrument's website to find it.

Instrument	Best used for	Format	Purchase required?	Training or qualification	Where to find it
APA Severity of Acute Stress Symptoms (NSESS acute stress scale)	Rating acute stress symptoms in adults and ages 11 to 17.	Self-report, 7 items	No (free from the APA for clinical use)	None beyond general clinical training	APA assessment measures
ASDS (Acute Stress Disorder Scale)	Acute stress symptoms within the first month after trauma.	Self-report, 19 items	No (free)	None beyond general clinical training	Original article
LEC-5 (Life Events Checklist)	Which potentially traumatic events a person has experienced.	Self-report, 17 items	No (free)	None beyond general clinical training	National Center for PTSD
PC-PTSD-5 (Primary Care PTSD Screen)	Quick screen for probable PTSD.	Self-report, 5 items	No (free)	None beyond general clinical training	National Center for PTSD
PCL-5 (PTSD Checklist for DSM-5)	Tracking symptoms and re-screening for PTSD after 1 month.	Self-report, 20 items, 5 to 10 minutes	No (free)	None beyond general clinical training	National Center for PTSD
C-SSRS (Columbia Suicide Severity Rating Scale)	Screening and assessing suicide risk; brief screener and full versions.	Clinician- or staff-asked questions, a few minutes	No (free)	Free online training offered and recommended	Columbia Lighthouse Project
AUDIT (Alcohol Use Disorders Identification Test)	Screening for drinking to cope, which raises risk after trauma.	Self-report or interview, 10 items	No (free)	None beyond general clinical training	World Health Organization
TSCYC (Trauma Symptom Checklist for Young Children)	Trauma symptoms in young children ages 3 to 12, by caregiver report.	Caregiver-report, 90 items (screening form 20)	Yes	★ Level B	PAR

★ **= requires professional qualification to purchase or specialized training.** Publishers sell these tests only to qualified users. At Pearson, **Level B** generally means a master's degree in a related field with formal assessment training, or a health care license; **Level C** means a doctorate in psychology or a related field with formal assessment training, or state licensure in a related field (Pearson qualifications policy). MHS, PAR, and WPS use similar levels. Check each publisher's page before ordering. "Original article" links go to the published study that introduced a free scale that has no official website; the scale can usually be obtained there or from its authors.

4. What works: treatment and the evidence

Bottom line for class: for people who meet ASD criteria, a few sessions of **trauma-focused CBT** **with exposure** reduce later PTSD. Single-session "debriefing" for everyone exposed is not recommended.

Evidence overview (honest effect sizes):

- **Exposure-based CBT:** in a randomized trial of 90 civilian trauma survivors with ASD, five weekly sessions of imaginal and real-life exposure left fewer people with PTSD after treatment (33%) than cognitive restructuring (63%) or waiting (77%). At 6 months, exposure also produced more full remissions (47% vs. 13% with cognitive restructuring) (Bryant et al., 2008).
- **Avoid routine debriefing:** across 11 trials, single-session psychological debriefing did not reduce distress or prevent PTSD; in one trial, people who were debriefed had **higher** PTSD risk a year later. The reviewers recommended stopping compulsory debriefing (Rose et al., 2002).
- **Medication (referral notes):** no medication is established for preventing PTSD after acute stress. A physician may treat severe sleep problems or other symptoms; routine benzodiazepine use is generally discouraged (general clinical consensus).

Trauma-focused CBT for ASD: how it is used (Bryant et al., 2008):

- **Psychoeducation and stabilization:** normal reactions, sleep, reconnecting with support, and reducing alcohol use.
- **Exposure:** gently and repeatedly recalling the memory in detail, and gradually returning to safe places and activities that are being avoided.
- **Identify and challenge trauma beliefs:** "The world is completely dangerous," "It was my fault," "I'm going crazy," "I will never be the same."

5. Religion, spirituality, and Christian integration: what the evidence shows

Many turn to faith after trauma. After the September 11 attacks, 90% of U.S. adults said they coped by turning to religion, and 98% by talking with others (Schuster et al., 2001).

How faith is used matters. Across 49 studies, positive religious coping (seeking God's help, finding spiritual meaning) went with better adjustment to stress, and negative religious coping (feeling punished or abandoned by God) with worse adjustment (Ano & Vasconcelles, 2005).

Intervention studies. We found **no trial of Christian or religiously integrated treatment for acute stress disorder** (literature search, November 1, 2026). Across 97 outcome studies of other conditions, therapy adapted to clients' religious beliefs worked about as well as standard therapy for distress and added spiritual benefits (Captari et al., 2018).

Honest summary for teaching:

1. Practical support for everyone, and brief exposure-based CBT for those with ASD, are the evidence base. Faith integration supports them; it does not replace them.
2. Most people turn to faith after disaster. Positive religious coping is worth encouraging; spiritual struggle ("God has abandoned me") deserves gentle attention.
3. Church crisis responses should offer comfort, practical help, and referral, not mandatory group "debriefing" sessions.

6. Practical Christian-integration notes

These apply Dr. Campbell's identify, challenge, replace-with-Scripture method within trauma-focused care. Clinical items rest on the research above. Pastoral items are theological application, not research findings.

- **Identify (clinical).** Listen for early trauma beliefs, including spiritual ones: "God abandoned me," "This happened because of my sin," "If I had more faith I wouldn't feel this way," "I'll never be safe again" (Ano & Vasconcelles, 2005).
- **Challenge (clinical).** Normalize early reactions, weigh the evidence for self-blame, and gradually approach safe reminders instead of avoiding them (Bryant et al., 2008). **Possible cognitive errors** to watch for, with their definitions from the Belief Counseling Cognitive Errors Checklist and the trauma symptoms they tend to feed:
  - **Hindsight bias** — I should have seen it coming. *Feeds:* guilt and replaying the event. *Sounds like:* "I should have seen it coming."
  - **Personalization** — taking blame for things outside my control. *Feeds:* self-blame. *Sounds like:* "It happened because of me."
  - **Catastrophizing** — expecting the worst possible outcome. *Feeds:* hypervigilance and startle. *Sounds like:* "I'll never be safe again."
  - **Overgeneralization** — one event becomes a rule. *Feeds:* fear and avoidance of reminders. *Sounds like:* "Nowhere is safe anymore."
  - **Emotional reasoning** — I feel it, so it must be true. *Feeds:* persistent fear. *Sounds like:* "I feel in danger, so I must be in danger."
  - **Avoidance** — pushing away painful thoughts. *Feeds:* avoidance of memories and reminders. *Sounds like:* "If I stay away from that road, I won't have to feel it."
  - **Fortune telling** — predicting a bad outcome. *Feeds:* negative mood and hopelessness. *Sounds like:* "I'll never be the same."
- **Questions to dispute the belief.** Have the client choose questions from the Questioning False Beliefs form that "speak to" the belief. Selected questions that fit the errors above:
  - "Is it possible to change the past?" *For:* hindsight bias.
  - "Am I being too hard on myself?" *For:* personalization and self-blame.
  - "What are the odds that the thing I dread will actually happen?" *For:* catastrophizing.
  - "Is it possible that I am distorting the facts?" *For:* overgeneralization.
  - "What's the actual evidence for my belief?" *For:* emotional reasoning.
  - "Am I underestimating my ability to cope?" *For:* avoidance.
  - "Has God given up hope that things will get better for me?" *For:* fortune telling.
  - "How might an all-loving God look at me or this situation?" *For:* the belief that God abandoned or is punishing me.
- **Replace (pastoral).** Offer Scripture of God's presence in suffering:
  - "God is our refuge and strength, an ever-present help in times of trouble." (Psalm 46:1)
  - "The LORD is near to the brokenhearted; He saves the contrite in spirit." (Psalm 34:18)
  - "Even though I walk through the valley of the shadow of death, I will fear no evil, for You are with me; Your rod and Your staff, they comfort me." (Psalm 23:4)
  - "Come to Me, all you who are weary and burdened, and I will give you rest." (Matthew 11:28)

- **Faith community (clinical).** Provide meals, transportation, childcare, and steady presence. Let people tell their story when they choose, without pressing for details or hurrying them to "move on."
- **Ministers:** avoid explaining the trauma as God's punishment, watch for those not improving after a few weeks, and refer to a trauma-trained clinician.

7. Quick classroom takeaways

- ASD means 9 or more of 14 trauma symptoms (intrusion, negative mood, dissociation, avoidance, arousal) lasting 3 days to 1 month after trauma. It predicts PTSD imperfectly; re-screen at 1 month.
- Brief exposure-based CBT reduces later PTSD in people with ASD; routine single-session debriefing does not help and may harm.
- Replace "God abandoned me" with the truth that He is "an ever-present help in times of trouble," offered alongside practical care.
- Screen early with the APA acute stress measure or ASDS, and re-screen with the PCL-5 after 1 month for PTSD.

Sources note

All 7 references were checked against Crossref, OpenAlex, and PubMed/Europe PMC records (Nov 1, 2026), and the figures quoted were checked against each study's abstract. Each DOI is a live link to the publisher record. Assessment instrument links and purchase and qualification levels were checked on the publishers' and distributors' websites, or on Crossref for original articles (Nov 1, 2026).

References

American Psychiatric Association. (2022). *Diagnostic and statistical manual of mental disorders* (5th ed., text rev.). https://doi.org/10.1176/appi.books.9780890425787

Ano, G. G., & Vasconcelles, E. B. (2005). Religious coping and psychological adjustment to stress: A meta-analysis. *Journal of Clinical Psychology*, *61*(4), 461–480. https://doi.org/10.1002/jclp.20049

Bryant, R. A. (2011). Acute stress disorder as a predictor of posttraumatic stress disorder: A systematic review. *The Journal of Clinical Psychiatry*, *72*(2), 233–239. https://doi.org/10.4088/JCP.09r05072blu

Bryant, R. A., Mastrodomenico, J., Felmingham, K. L., Hopwood, S., Kenny, L., Kandris, E., Cahill, C., & Creamer, M. (2008). Treatment of acute stress disorder: A randomized controlled trial. *Archives of General Psychiatry*, *65*(6), 659–667. https://doi.org/10.1001/archpsyc.65.6.659

Captari, L. E., Hook, J. N., Hoyt, W. T., Davis, D. E., McElroy-Heltzel, S. E., & Worthington, E. L., Jr. (2018). Integrating clients' religion and spirituality within psychotherapy: A comprehensive meta-analysis. *Journal of Clinical Psychology*, *74*(11), 1938–1951. https://doi.org/10.1002/jclp.22681

Rose, S., Bisson, J., Churchill, R., & Wessely, S. (2002). Psychological debriefing for preventing post traumatic stress disorder (PTSD). *Cochrane Database of Systematic Reviews*, *2002*(2), CD000560. https://doi.org/10.1002/14651858.CD000560

Schuster, M. A., Stein, B. D., Jaycox, L. H., Collins, R. L., Marshall, G. N., Elliott, M. N., Zhou, A. J., Kanouse, D. E., Morrison, J. L., & Berry, S. H. (2001). A national survey of stress reactions after the September 11, 2001, terrorist attacks. *New England Journal of Medicine*, *345*(20), 1507–1512. https://doi.org/10.1056/NEJM200111153452024

Prepared for Clinical Companion (Brian Campbell). Teaching overview only. Clinically review before use. Not a substitute for the DSM-5-TR or clinical judgment. All 7 references and DOIs verified via Crossref, OpenAlex, and PubMed/Europe PMC records (Sunday, Nov 1, 2026). Effect sizes as reported in the cited sources.